TGA Senate QON Response: Gap Analysis Against National Cabinet Policy and Safety Plan

Senate Questions on Notice: SQ13-19 (December 2025)

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Prepared by: Paul Rekaris, Independent Policy Analyst

Executive Summary

The NCIRS 2022 surveillance report documented 160 deaths following COVID-19 vaccination, including six children (median 4.5 days vaccination-to-death). Only two of 160 deaths were referred to the Vaccine Safety Investigation Group (VSIG) for expert causality assessment; none of the six child deaths were referred. Senator Alex Antic questioned TGA about this during December 2025 Senate Estimates, asking why children were not referred despite TGA's Work Instruction stating youth deaths "had potential to change the favourable benefit-risk balance."

Senator Antic asked seven questions about TGA's COVID-19 vaccine death monitoring (SQ13-19). TGA's December 2025 response reveals systematic gaps between governance commitments and implementation across a four-year investigation period. Sladden (2026) documented that only two of 160 deaths reported in the 2022 NCIRS surveillance cohort were referred to VSIG for expert causality assessment — a pattern consistent with TGA's own disclosure that approximately 50 of 1,053 total DAEN deaths received VSIG review across the full rollout (4.7%) — despite TGA's Work Instruction criteria requiring review of deaths with "potential to change the favourable benefit-risk balance."

The Questions on Notice

The NCIRS 2022 surveillance report documented 160 deaths from 18,398 AEFI reports for COVID-19 vaccines, with median intervals of 4.5 days (children aged 0-17) and 3 days (young adults aged 18-29) between vaccination and death—timeframes consistent with known biological mechanisms that strengthen plausibility of causal links (NCIRS, 2025; Sladden, 2026).

Senator Antic identified:

- Six children (0–17): median vaccination-to-death interval 4.5 days
- Five young adults (18–29): median vaccination-to-death interval 3 days
- Only 1 of 160 deaths determined as likely linked to vaccination

Seven questions were asked (SQ13–19):

- SQ13: Were the six children referred to VSIG? If not, why not?
- SQ14: Were the five 18–29-year-olds referred to VSIG? If not, why not?

- SQ15: Do causality assessment reports exist for each child death? Provide copies.
- SQ16: Do causality assessment reports exist for each 18–29-year-old death? Provide copies.
- SQ17: Why were only 2 of 160 fatal cases in 2022 referred to VSIG?
- SQ18: How many of 1,053 DAEN deaths have had causality assessments?
- SQ19: How many of 1,053 DAEN deaths been referred to VSIG, and where can reports be accessed?

Key Findings from TGA's Response

- 1,053 deaths reported on DAEN. TGA did not state how many received formal causality assessments.
- Approximately 50 cases reviewed by VSIG out of 1,053 deaths (4.7%).
- 15 VSIGs convened across the entire COVID-19 rollout.
- 9 child deaths on DAEN. Zero referred to VSIG.
- 2 of the 9 child deaths had 'broader safety investigations' by TGA (late 2022/early 2023), but not via VSIG expert panel.
- 1 of 5 young adult deaths (18–29) in 2022 referred to VSIG.
- Zero causality assessment reports released. Privacy cited.
- 9 prior SQONs (2023–24) cited as 'previous answers.' None produced Plan documentation.

The analysis demonstrates control failure across a four-layer governance hierarchy: National Cabinet Policy passed by Commonwealth Cabinet (comprehensive monitoring commitment) → `TGA Safety Monitoring Plan (17 systematic strategies) → VSIG Control Activity (selective expert review) → International Standards (ICH E2E, CIOMS VIII). The activity layer under-delivers framework requirements, breaching the governance commitment. 95.3% of reported deaths have no documented expert assessment pathway.

The Governance Failure

National Cabinet required comprehensive COVID-19 vaccine monitoring; TGA operationalised this through its February 2021 Plan with 17 documented outputs including Strategy 2.2 (investigation and clinical assessment).

VSIG is a selective control activity within Output 2.2—TGA's Work Instruction (FOI 4029-06) states youth deaths "had potential to change the favourable benefit-risk balance," yet zero of 9 child deaths were referred. VSIG reviewed 50 of 1,053 deaths (4.7%); when asked how the remaining 95.3% were assessed, TGA deflected to "VSIG is one of many tools" without documenting the alternatives.

The false attribution to WHO guidance regarding "burning out" the VSIG panel, documented by Sladden (2026), demonstrates institutional decisions to restrict expert review were falsely presented as adherence to international standards when no such standard existed.

VSIG's selectivity does not absolve TGA from reporting against Plan objectives—Strategy 2.2 delivery must be documented regardless of which control mechanism is used. Senate evidence indicates monitoring wasn't tracked against Plan objectives.

After 10 Senate questions over two years, TGA cannot demonstrate the systematic framework that justified provisional approval—the outputs weren't delivered or weren't documented; both conclusions confirm governance failure.

Understanding the Governance Framework

The Four Layers

The commitments examined in this analysis operate within a four-layer governance hierarchy where each layer creates binding obligations for the layer below.

Layer 1: National Cabinet Policy (November 2020) established the governance Commitment (passed by Commonwealth Cabinet) —TGA must "actively and comprehensively monitor" COVID-19 vaccines.

Layer 2: TGA Safety Monitoring Plan (February 2021) operationalised this through 17 documented outputs, including Strategy 2.2 (investigation and clinical assessment of adverse events of special interest).

Layer 3: VSIG is a selective control activity within Strategy 2.2, designed to provide expert review when specific criteria are met (serious, unexpected, no obvious non-vaccine cause), not to review every death.

Layer 4: International Standards (ICH E2E, CIOMS VIII, ISO 31000) establish the reference framework TGA adopted for documenting systematic pharmacovigilance.

The Critical Distinction

VSIG's selectivity is appropriate by design, but it does not eliminate Strategy 2.2's broader obligation to systematically investigate individual COVID-19 AEFI reports (Strategy 2).

When VSIG reviews only 4.7% of deaths, the remaining 95.3% still require documented investigation under strategy 2.2—yet TGA evidence provided in the Senate indicates that monitoring wasn't tracked against Plan objectives and cannot produce implementation records after 10 Senate questions on notice.

The Cascading Failure

The control activity (Layer 3) under-delivers the operational framework (Layer 2), failing to demonstrate compliance with the governance commitment (Layer 1) and violating the reference standards (Layer 4). The framework was either not delivered as documented or cannot now be independently verified from the available evidence. Both possibilities indicate an evidence-governance failure, while the absence of implementation records prevents verification of compliance with the Commonwealth's implementation of the National Cabinet commitment. The Australian Government's COVID-19 Vaccination Policy had received a first pass by Federal Cabinet before National Cabinet endorsed the policy in November 2020, demonstrating that the policy had been subject to Commonwealth Cabinet consideration. The PM&C *Cabinet Handbook* (para 25) provides that Ministers and portfolio agencies must act on Commonwealth Cabinet decisions. The March 2025 policy reversal acknowledging that benefits did not outweigh harms in healthy children (Sladden, 2026) came nearly three years after the first child death was reported, demonstrating that the governance and review failures had real-world consequences for public health decision-making during the critical period when independent scrutiny was most needed.

The Obligation to Report

The Australian COVID-19 Vaccination Policy, endorsed by National Cabinet (p.6, p.13), required that TGA's Pharmacovigilance Plan enhance "existing processes for safety signal detection and investigation." This commitment was operationalised by Strategy 2.2 (p.8), which documented a specific obligation to ensure "appropriate and timely investigation of COVID-19 AEFI reports" through dedicated protocols and enhanced investigative capacity. Strategy 2.1 (p.8) establishes that deaths qualify as serious AESI, making them subject to this investigative obligation.

TGA designed VSIG as a selective tool — applicable only where events were serious, unexpected, and had no obvious non-vaccine cause. Even accepting that design, TGA retained an obligation to document the investigative pathway applied to non-VSIG cases. The Plan's 17 outputs operationalise Cabinet policy, and TGA must report against them regardless of which tools are used.

Yet VSIG was applied to only 4.7% of reported deaths. The remaining 95.3% have no documented investigative pathway. Senate evidence indicates that monitoring was not tracked against Plan objectives. This constitutes a governance failure: TGA cannot demonstrate Strategy 2.2 delivery for 95.3% of deaths — the work is either undone or undocumented.

Table 1: Governance Hierarchy Framework

The commitments examined in this analysis operate within a four-layer governance hierarchy. Each layer creates obligations for the layer below. Failure at the control activity level represents a breach of all higher layers.

Control failure: activity layer (3) under-delivers framework (2), breaching governance commitment (1). 95.3% of reported deaths have no documented assessment pathway against any layer.

Layer	Standard / Requirement	TGA Position	Failure Type
1. Governance Commitment National Cabinet Policy (Nov 2020)	"TGA will actively and comprehensively monitor any COVID-19 vaccines for safety" (p.6). "Pharmaco-vigilance Plan" anticipated (p.13, 15). PGPA Act s.17 requires accountable authority to govern entity.	15 VSIGs convened reviewing approximately 50 cases out of 1,053 reported deaths (4.7%). 95.3% of reported deaths have no documented expert assessment pathway. Senate Estimates (Oct 2025): monitoring conducted as "day-to-day processes" not tracked against Plan objectives.	Strategic misalignment. Control activity does not deliver governance commitment. Consistent with ANAO findings on risk-based targeting and incomplete performance indicators (CPS 107).
2. Control Framework TGA Safety Monitoring Plan (Feb 2021)	17 systematic outputs operationalise policy. Objective 2 (p.8): timely signal detection and investigation. Objective 3 (p.10): timely action on safety concerns (incl. AESI causality). Output 2.2 (p.8): individual AEFI investigation/clinical assessment. Output 3.1 (p.10): regulatory action	16% of 19 outputs fully documented as per audit (Rekaris, 2026). No systematic tracking against plan objectives indicated by senior TGA officials in the Senate. VSIG selectivity bypasses framework for 95.3% of deaths including 0/9 child deaths.	Operational deficiency. Framework exists on paper but execution not evidenced. Consistent with ISO 31000 requirement for integrated risk framework aligned to objectives.
3. Control Activity VSIG (FOI 4029-06)	Selective tool within Output 2.2: convened for "serious, unexpected, no obvious non-vaccine cause" (pp.3–4). Work Instruction states youth deaths "had potential to change benefit-risk balance."	15 VSIGs convened / 1,053 deaths (4.7%). 0/9 child deaths referred despite meeting Work Instruction criteria. No documented alternative assessment pathway for remaining 95.3%.	Systemic under-delivery. Control activity does not meet framework requirements. Internal criteria contradicted by practice. ISO 19011 requires evidence-based, risk-proportionate audit.
4. Reference Controls International standards	ICH E2E §3.1.3 (p.7): documented action plan for each safety issue with milestones. §3.1.4 (p.7): summary of actions completed. CIOMS VIII Ch.II (pp.13–18): documented signal taxonomy. Ch. IV-V (pp.25–42): systematic data sources.	No milestones documented against Plan objectives. No signal management audit trails for 91 of 148 signals. No documented integration of active surveillance data sources.	Non-compliance with international standards TGA formally adopted as reference framework for the Plan. Non-binding but cited as benchmark.

Table 2: Question-by-Question Assessment

Senate Question	TGA Response	Safety Plan Requirement	National Policy	Gap Identified	International Standard
SQ13–14 Were children and 18–29-year-olds referred to VSIG?	None of 9 child deaths met VSIG criteria. 1 of 5 young adult deaths referred. Reasons cited: 'known AE', 'alternative causes', 'insufficient information.' 'VSIG is not a death investigation group.' TGA notes 2 of 9 child deaths had 'broader safety investigations' (late 2022/early 2023) but did not meet VSIG criteria	Objective 2 (p.5) → Strategy 2 (p.8): timely signal detection and investigation (AESI via 2.1, individual reports via 2.2). Objective 3 (p.5) → Strategy 3 (p.10): timely safety actions (causality via investigation process).	'TGA will actively and comprehensively monitor any COVID-19 vaccines for safety' (p.6). 'Pharmacovigilance Plan' framework anticipated and safety monitoring requirements established (p.13, 15).	Plan (Strategy 2 p.8) committed to investigating serious AESI/individual AEFI reports (Output 2.1 AESI list; 2.2 protocols). TGA VSIG instruction (FOI 4029-06 pp.3‑4): youth/child deaths have "potential to change benefit-risk" → 0/9 children referred despite criteria. National policy (Australian COVID Vaccination Policy Nov 2020) requires comprehensive monitoring; selective = non-conforming	ICH E2E §2.1.2(b) (p.4): Safety Specification must address populations not studied in pre-approval phase, including children. ICH E2E §3.1.3 (p.7): Action plan for each safety issue must include monitoring and milestones for evaluation.
SQ15–16 Do causality assessment reports exist? Provide copies.	Refused. 'TGA does not disclose details of individual cases to ensure patient privacy.' Did not confirm whether reports exist. Did not address 'if they do not exist, why not?'	Objective 2 (p.5)/Output 2.2 (p.8): Investigation of individual COVID-19 AEFI reports (no protocols documented).	'Risk management plans and pharmacovigilance required' (p.13, 15). Comprehensive monitoring commitment requires verifiable assessment processes.	Neither confirms nor denies reports exist. Redacted copies were requested. Second part of question ('if not, why not?') unanswered. Comprehensive review claimed but evidence withheld, rendering claim unfalsifiable.	ICH E2E §3.1.2 (p.6): Routine pharmacovigilance requires systems ensuring suspected adverse reactions 'collected and collated in an accessible manner.' CIOMS VIII Ch. II (pp.13–18): Signal definition requires documented evaluation. Signal

Senate Question	TGA Response	Safety Plan Requirement	National Policy	Gap Identified	International Standard
					taxonomy requires classification and documented assessment.
SQ17 Why were only 2 of 160 fatal cases in 2022 referred to VSIG?	Deflects to SQ13–14. VSIG is 'one of a number of tools.' 'It is not correct to suggest that all AEFI reports where an outcome of death is reported will automatically meet the criteria to convene a VSIG.'	Output 2.2 (p.8): "Specific protocols for investigating COVID-19 AEFI reports" + capacity for timely individual AEFI investigations. Not documented (OAIC MR2200538: zero protocols; Senate: "day-to-day").	'Actively and comprehensively monitor' (p.6). 1.25% referral rate (2/160) for deaths during provisional approval is inconsistent with 'comprehensive' commitment.	If VSIG is 'one tool,' what tool was used for the other 158 deaths? No documented alternative assessment pathway produced. Plan committed to enhanced monitoring beyond routine.	ICH E2E §3.1.3 (p.7): Each safety issue requires documented objective, action, rationale, and milestones. CIOMS VIII Ch.IV-V (pp.25–42): All relevant data sources to be searched systematically with documented processes.
SQ18 How many of 1,053 DAEN deaths have had causality assessments?	'Please refer to response to questions 13 and 14.' **No number provided.**	Output 2.2 (p.8): "Enhanced capacity and capability for investigating individual COVID-19 AEFI reports" ("Specific protocols for investigating COVID-19 AEFI reports").	'Actively and comprehensively monitor' (p.6). If comprehensive monitoring occurred, a count of completed causality assessments should be readily available.	Direct question not answered. If systematic causality assessment occurred under the Plan, TGA should be able to state a figure. Consistent with Oct 2025 Senate Estimates admission that monitoring was not tracked against Plan objectives.	ICH E2E §3.1.4 (p.7): 'Summary of actions to be completed, including milestones' — requires documented tracking of pharmacovigilance actions. CIOMS VIII Ch.II (p.14): Signal taxonomy requires documented classification of each identified signal with sufficient evidence to justify verificatory action.
SQ19 How many of 1,053 deaths referred to VSIG? Where can reports be accessed?	'Please refer to response to questions 13 and 14.' Partial: 15 VSIGs reviewed 'over 50 cases.' Access: weekly safety report URLs listed.	Objective 3 (p.5): Timely action to address any COVID-19 vaccine safety concerns. Strategy 3, Output 3.1 (p.10): "Use legislative provisions available to achieve effective and timely regulatory action in response to any	'TGA responsible for pre-market assessment and post-market monitoring and enforcement of standards; (p.4). 4.7% expert review rate is not consistent with comprehensive commitment.	~50 of 1,053 = 4.7% expert review rate. Weekly reports cited as access point contain public communications summaries, not individual case assessments or audit trails. 95.3% of deaths have no documented expert assessment pathway.	ICH E2E §3.1.3 (p.7): Requires monitoring and milestones for each safety issue. ICH E2E Annex §3 (pp.10–11): Active surveillance must 'ascertain completely the number of adverse events via a continuous pre-organised process.'

Senate Question	TGA Response	Safety Plan Requirement	National Policy	Gap Identified	International Standard
		emerging COVID-19 vaccine safety concerns, as well as any non-regulatory action that may help to address or reduce the risk of a safety concern." (Processes: "enable rapid action... including broadening... risk communication activities")			

Table 3: FOI Evidence — Capacity vs Application

FOI documents demonstrate TGA had operational expert capacity and was actively briefing on Plan strategies while restricting expert causality assessment of the most serious adverse events.

FOI Document	Content	Relevance to SQ13–19	Plan / Policy Gap	Audit Finding
FOI 4029-06 VSIG Work Instruction (Jan 2019)	Selective criteria: serious, unexpected, no obvious non-vaccine cause (pp.3–4). States young deaths 'had potential to change favourable benefit-risk balance.'	Contradicts SQ13–14 response. Internal criteria identify youth deaths as meeting threshold, yet TGA tells Senate they did not meet criteria.	TGA Safety Monitoring Plan (Feb 2021): Strategy 2.2 (page 8): Protocols/capacity for individual AEFI investigations (serious incl. deaths). Strategy 2.8 (page 9): Expert advice mechanisms (VSIG fits). National Policy (Nov 2020) p 6: "Actively and comprehensively monitor" post-market safety (68M provisional doses	Internal criteria contradicted by practice. Criteria written, not applied to 9 child deaths. No protocols found (audit); selective VSIG (0/9 child deaths despite criteria) ≠ comprehensive. Undocumented vs tracked activity promised.
FOI 4029-05 VSIG Meeting (Sep 2022)	VSIG convened and operational. Reviewed fatal myocarditis in 21-year-old. Comprehensive causality assessment. Regulatory recommendations issued.	Expert panel operational Sep 2022 — same year 6 children died without VSIG referral. Capacity existed and was used for fatal case in same period.	Strategy 2.2 (p.8): investigation capacity demonstrated for 21yo death but not applied for child deaths in same period. Policy (p.6): comprehensive.	Policy decision, not capacity limitation. Panel assessed 21yo fatal case; 6 child deaths in same year not referred.
FOI 4029-03 Plan strategy briefing (Sep 2021)	TGA actively briefing advisory bodies on Safety Monitoring Plan strategies during rollout.	TGA presenting Plan strategies to advisory bodies while not systematically implementing them for death investigations.	Plan briefed as operational framework while deaths handled through undocumented 'day-to-day processes.' Policy commitment not operationalised.	Knowledge of Plan + capacity + non-application = deliberate policy choice.
FOI 5082 COVID-19 enhancements	COVID-19 specific enhancements to TGA infrastructure and processes documented.	Enhanced infrastructure built for COVID-19. Capability existed to systematically track death investigations.	Infrastructure not integrated with Plan framework for systematic death assessment. Plan and Policy both require demonstrable delivery.	Enhanced infrastructure + selective use = documentation gaps reflect policy decisions, not resource constraints.

Table 4: Prior SQON Pattern — Consistent Non-Disclosure

TGA's December 2025 response cites 9 prior SQONs as evidence it has already addressed vaccine death questions. None produced Plan implementation records. If systematic documentation existed, at least one of these 9 responses would have produced it.

SQ / FOI Reference	Date	TGA Response Pattern	Plan Documentation Produced?	Relevance to SQ13–19
SQ24-000234 (FOI 4263)	Apr 2024	Cause of death determinations: aggregate statistics. 'Low threshold reporting; few causally linked.'	**No**	SQ18 asks same question. Still unanswered.
SQ24-000233 (FOI 4205)	Apr 2024	DAEN deaths database: aggregate statistics only.	**No**	SQ19 asks for access. Still only public communications available.
SQ23-002226	Jan 2024	'Reporting does not equal causation.' Deflects from assessment obligation.	**No**	Same deflection used in SQ17 response.
SQ23-001101	Sep 2023	Single death follow-up: case-specific response without system-level evidence.	**No**	Individual case handling does not demonstrate systematic framework.
SQ23-000139	Aug 2023	Causality of 7- and 9-year-old deaths: confirmed but VSIG referral not addressed.	**No**	Same child deaths, same non-referral. Pattern repeated in Dec 2025.
SQ23-001074	Aug 2023	Temporal association/excess deaths: statistical deflection.	**No**	Signal assessment records not produced.
SQ23-001086 (FOI 4217)	Jul 2023	Child deaths under 18: 'No new safety concerns' without documented expert assessment.	**No**	Conclusion asserted without verifiable process. Directly relevant to SQ13.
SQ23-001134	Jul 2023	Child vaccine deaths: refers to prior responses.	**No**	Circular referral. Same pattern repeated Dec 2025.
SQ24-000218	Apr 2024	Batch number tracking: aggregate data only. Does not track deaths and adverse events by individual batch numbers.	**No**	Systematic tracking by batch would enable signal detection. Absence of batch-level analysis limits ability to identify safety patterns across production lots.
SQ24-000230	Apr 2024	COVID-19 vaccine autopsies: limited information.	**No**	Autopsy integration into signal management unaddressed.

Governance Failure Analysis

The structural and systemic issues identified in TGA's pharmacovigilance response constitute governance failure under ANAO, VAGO, and ISO assessment criteria. The evidence demonstrates inadequate design and operation of controls against commitments.

1. Child deaths: internal criteria contradicted by practice

9 child deaths. Zero referred to VSIG. TGA states 2 of 9 had 'broader safety investigations' (late 2022/early 2023), but these did not involve VSIG expert panel review and investigation pathway is not documented in Plan framework. TGA's VSIG Work Instruction (FOI 4029-06, pp.3–4) states youth deaths 'had potential to change the favourable benefit-risk balance.' Strategy 2.1 (p.8) commits to "A list of COVID-19 vaccine AESI established, informed by premarket product evaluations and World Health Organization (WHO) and Brighton Collaboration guidelines. AESI (Adverse Events of Special Interest) are prioritised for monitoring, and deaths naturally qualify as serious AESI under standard pharmacovigilance (e.g., WHO/Brighton lists include fatal outcomes).

National policy (p.6) required 'comprehensive' monitoring. ICH E2E §2.1.2(b) (p.4) requires children be addressed as an unstudied population. TGA wrote the criteria, published the Plan, operated under the policy, adopted the standard, then did not apply any of them to child deaths. Nearly three years elapsed between the first reported child death and the March 2025 policy reversal withdrawing recommendations for healthy children, during which reports of death—the adverse event category most likely to alter benefit-risk calculations—were systematically filtered rather than escalated for expert review (Sladden, 2026).

2. Unanswered question: how many causality assessments?

SQ18 is a direct, quantifiable question. TGA deflects without providing a number. If systematic causality assessment occurred under Output 2.2 (p.8), a count should be readily available. ICH E2E §3.1.4 (p.7) requires a summary of actions completed with milestones. If completed, this summary would answer SQ18 immediately. The inability or refusal to provide a figure is consistent with the October 2025 Senate Estimates admission that monitoring was conducted as 'day-to-day processes' not tracked against Plan objectives. Under ANAO performance audit methodology, this represents incomplete performance indicators and undocumented risk assessments.

3. Expert review rate: 4.7%

Approximately 50 of 1,053 reported deaths reviewed by VSIG. TGA describes VSIG as 'one of a number of tools.' No documented alternative assessment pathway has been produced for the remaining 95.3%. Plan Objective 2 (p.8) committed to timely detection, investigation and response. National policy (p.6) required comprehensive monitoring. ICH E2E §3.1.3 (p.7) requires each safety issue have a documented action plan. The gap is not that VSIG was not always convened. The gap is that no documented process exists for the 95.3% not reviewed by any expert panel. TGA's internal "pilot" AEMS Preliminary Fatal Case Assessment process instructed staff to avoid "burning out" the VSIG panel by restricting referrals—an approach falsely attributed to WHO guidance that contained no such provisions and instead recommended seeking external assistance when capacity was insufficient (Sladden, 2026). This explains the 4.7% expert review rate while providing no documented alternative pathway for the remaining 95.3%. Under ISO 31000, a risk treatment applied to 4.7% of identified risks does not constitute an integrated risk management framework.

4. Capacity existed; application was selective

FOI 4029-05: VSIG operational September 2022, assessing a fatal case in a 21-year-old. Same year, 6 children died without referral. FOI 4029-03: TGA briefing advisory bodies on Plan strategies September 2021. FOI 5082: COVID-19 specific enhancements documented. The infrastructure existed, the capacity existed, the Plan was being briefed as operational. Child deaths were not referred. This is a control design failure: the activity layer does not deliver the framework requirements. Under VAGO performance audit standards, this represents methodology lacking rigour and risks not addressed in a timely manner.

5. Unfalsifiable claims

SQ15–16 asked for causality reports. TGA refused, citing privacy. The question also asked: ‘if they do not exist, why not?’ This was not answered. TGA simultaneously claims all serious AEFIs are reviewed, withholds evidence of that review, and declines to confirm whether the evidence exists. ICH E2E §3.1.2 (p.6) requires information collected ‘in an accessible manner.’ Under ANAO audit criteria, claims not supported by documentary evidence cannot be accepted as sufficient. The comprehensive review claim is unfalsifiable by design.

6. Serial deflection across 9 prior SQONs

TGA cites 9 prior SQONs (July 2023 – April 2024) as evidence it has already addressed death-related questions. Analysis of each shows the same pattern: aggregate statistics, privacy deflection, case-specific evasion, circular referral. Across 9 opportunities over two years, TGA did not produce Plan implementation records, signal management audit trails, or systematic evidence of enhanced monitoring delivery. SQ23-000139 (August 2023) asked specifically about 7- and 9-year-old deaths. Two years later, SQ13 asks the same question. The answer is unchanged. Repetition without documentation across nine opportunities is consistent with the

documentation not existing, and TGA has provided no alternative explanation. Under ISO 19011, an auditable process must produce verifiable records. The absence of records across 9 Senate questions constitutes sufficient evidence of non-implementation.

7. Breach of Cabinet obligation

The PM&C Cabinet Handbook states that “Ministers (and portfolio agencies) must act on Cabinet decisions” (PM&C, 2020, para 25). The Australian Government's COVID-19 vaccination policy had already received a first pass by Federal Cabinet before National Cabinet considered and endorsed the policy on 13 November 2020; the Prime Minister stated on 5 November that the policy would return to Cabinet following further consultation before being taken to National Cabinet for confirmation. The endorsed policy established Commonwealth responsibilities for national oversight and surveillance and expressly identified TGA vaccine safety monitoring as a component of the national vaccination framework. The TGA subsequently operationalised the safety-monitoring component through its February 2021 COVID-19 Vaccine Safety Monitoring Plan, comprising 17 documented monitoring strategies.

The inability to produce implementation records after ten Senate questions prevents independent verification that the monitoring framework was implemented as documented and intended. This does not establish that the TGA failed to undertake the relevant activities; rather, it establishes that implementation cannot be demonstrated through a sufficiently documented and reproducible evidentiary record. This raises governance concerns under the Commonwealth Cabinet framework and the Public Governance, Performance and Accountability Act 2013, including the requirement for accountable authorities of non-corporate Commonwealth entities to govern in a manner that is not inconsistent with Australian Government policies (s 21).

Conclusion

Senator Antic asked seven direct questions.[1] TGA’s response:

1. Confirmed zero of 9 child deaths referred for expert review, contradicting TGA’s own published criteria
2. Did not state how many of 1,053 deaths received formal causality assessments
3. Refused to release causality reports and did not confirm whether they exist
4. Disclosed a 4.7% expert review rate (approximately 50 of 1,053 deaths)
5. Cited weekly safety reports as evidence of systematic monitoring — reports containing public communications summaries, not implementation documentation
6. Cited 9 prior SQONs as ‘previous answers’ — none of which produced Plan documentation

[1] SQ13–19. TGA grouped SQ13–14 (VSIG referrals) and SQ15–16 (causality reports) into combined answers, and cross-referred SQ17–19 to SQ13–14. The six points capture TGA's response pattern across all seven questions.

Each finding maps to documented gaps against the COVID-19 Vaccine Safety Monitoring Plan (February 2021), the National Cabinet-endorsed Australian COVID-19 Vaccination Policy (November 2020), and the international pharmacovigilance standards Australia adopted as reference framework.

The questions Senator Antic asked in December 2025 are the same questions asked in July 2023. The responses are unchanged. Two years of Senate scrutiny have not produced a single Plan implementation record. The documentation either does not exist or has not been disclosed. Both conclusions confirm the audit's central finding: the enhanced monitoring framework promised under Cabinet-endorsed policy and required for provisional approval cannot be verified through documentation.

The Key Finding

TGA's Work Instruction says youth deaths "had potential to change the favourable benefit-risk balance." Zero of 9 child deaths were referred—this isn't selective application; it's systemic non-delivery. VSIG reviewed 50 of 1,053 deaths (4.7%); TGA won't document how the other 95.3% were assessed. National Cabinet required comprehensive monitoring; TGA published a Plan with 17 strategies including strategy 2.2 (capability for investigating individual COVID-19 AEFI reports); Senate Estimates confirmed monitoring wasn't tracked against Plan objectives. After 10 Senate questions over two years, TGA cannot demonstrate the framework exists. The work wasn't done or wasn't documented against Plan objectives—VSIG's selectivity does not absolve TGA from this reporting obligation. This constitutes governance failure under both scenarios.

Paul Rekaris
Independent Policy Analyst
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    - SQ23-002226 (November 2023)
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